

ATROPINE SULFATE/DEMO

Solution for injection 1 mg/ml glass amps

1. DESCRIPTION OF THE MEDICINAL PRODUCT

1.1 Name of the medicinal product: ATROPINE SULFATE/DEMO

1.2 Qualitative and Quantitative Composition: Each ampoule of 1ml contains 1mg atropine sulfate.

1.3 Pharmaceutical form: Sterile aqueous solution for parenteral administration.

1.4 Description - Packaging: The specialty circulates in ampoules of colourless glass of 1st hydrolytic class which contain clear, colourless and odourless solution and in carton box of 50 or 100 ampoules.

1.5 Pharmacodynamic properties: Atropine is an antimuscarinic agent which competitively antagonizes acetylcholine at postganglionic nerve endings, thus affecting receptors of the exocrine glands, smooth muscle and the central nervous system. Peripheral effects include tachycardia, decreased production of saliva, sweat, bronchial, nasal, lachrymal and gastric secretions decreased intestinal motility and inhibition of micturition. Atropine increases sinus rate and sinoatrial and AV conduction. Usually heart rate is increased but there may be an initial bradycardia. Atropine inhibits secretions throughout the respiratory tract and relaxes bronchial smooth muscle producing bronchodilation.

1.6 Marketing Authorization Holder: DEMO S.A., Pharmaceutical Industry, 21 Km National Road Athens-Lamia, 14568 Kryoneri, Athens, Greece. Tel: +30 210 8161802, Fax: +30 2108161587.

2. WHAT YOU SHOULD KNOW ABOUT THE MEDICINE YOUR DOCTOR HAS PRESCRIBED TO YOU

2.1 Therapeutic Indications: Acute myocardial infarction with AV conduction block due to excess vagal tone (Wenkebach Type I, second-degree AV block) and sinus bradycardia, with associated hypotension and increased ventricular irritability. Atropine can also be used in cardiopulmonary resuscitation for the treatment of sinus bradycardia accompanied by hypotension, hypoperfusion or ectopic arrhythmias. Parenteral atropine is indicated as an antisialogogue in anaesthetic premedication to prevent or reduce secretions of the respiratory tract. During anaesthesia, atropine may be used to prevent reflex bradycardia and restore cardiac rate and arterial pressure resulting from increased vagal activity associated with laryngoscopy, tracheal intubation and intra-abdominal manipulation. It may also be administered to block muscarinic effects when neostigmine is used to counteract muscle relaxants such as tubocurarine. Parenteral atropine is an antidote for cardiovascular collapse following overdose of anticholinesterases; in the treatment of poisoning from organophosphorous insecticides or from chemical warfare 'nerve' gases and in the treatment of mushroom poisoning.

2.2 Contraindications: Contra-indications are not applicable to the use of atropine in life threatening emergencies (eg. asystole). Atropine is contraindicated in patients with known hypersensitivity to the drug, obstruction of the bladder neck eg due to prostatic hypertrophy, reflux oesophagitis, closed angle glaucoma, myasthenia gravis (unless used to treat the adverse effects of an anticholinesterase agent), paralytic ileus, severe ulcerative colitis and obstructive disease of the gastrointestinal tract.

2.3 Special warnings and precautions for use

2.4.1 General: Antimuscarinic agents should be used with caution in the elderly and children since these patients may be more susceptible to adverse effects. Atropine should also be used with caution in patients with hyperthyroidism, hepatic or renal disease or hypertension. Use with caution in febrile patients or when ambient temperature is high since antimuscarinics may cause an increase in temperature. Antimuscarinics block vagal inhibition of the SA nodal pacemaker and should thus be used with caution in patients with tachyarrhythmias, congestive heart failure or coronary heart disease. Parenterally administered atropine should be used cautiously in patients with chronic pulmonary disease since a reduction in bronchial secretions may lead to formation of bronchial plugs. Antimuscarinics should be used with extreme caution in patients with autonomic neuropathy.

Antimuscarinics decrease gastric motility, relax the lower oesophageal sphincter and may delay gastric emptying; they should therefore be used with caution in patients with gastric ulcer, oesophageal reflux or hiatus hernia associated with reflux oesophagitis, diarrhoea or GI infection.

2.4.2 Pregnancy and Lactation: Atropine crosses the placenta. Studies in humans have not been done and only limited information is available from animal studies. Intravenous administration of atropine during pregnancy or at term may cause tachycardia in the foetus. Atropine should only be administered to pregnant women if the benefits outweigh the risks to the foetus. Trace amounts of atropine appear in the breast milk and may cause antimuscarinic effects in the infant; lactation may be inhibited.

2.4.3 Effects on ability to drive and use machines: Not applicable; this preparation is intended for use only in emergencies.

2.5 Interaction with other medicinal products and other forms of interaction: The effects of atropine may be enhanced by the concomitant administration of other drugs with anticholinergic activity eg. tricyclic antidepressants, antispasmodics, anti-parkinsonian drugs, some antihistamines, phenothiazines, disopyramide and quinidine. By delaying gastric emptying, atropine may alter the absorption of other drugs.

2.6 Posology and method of administration:

Adults, children over 12 and the elderly:

Bradyarrhythmias: intramuscular or intravenous, **300 to 600 mcg (0.3 to 0.6 mg)** every four to six hours to a total dose of 2 mg. In cardiac resuscitation, intravenous **500 mcg (0.5 mg)** repeated at **5 minute** intervals until the desired heart rate is achieved. In asystole, **3 mg** may be given intravenously as a once only single dose. If atropine cannot be administered intravenously during resuscitation, **2-3** times the intravenous dose may be administered via an endotracheal tube.

Premedication before anaesthesia: intramuscular or subcutaneous, 300 to 600 mcg (0.3 to 0.6 mg) 30- 60 minutes before surgery or the same dose intravenously immediately before surgery.

To control muscarinic side effects of neostigmine: intravenous, 600 to 1200 mcg (0.6-1.2 mg).

Anticholinesterase poisoning: intramuscular or intravenous, 1 to 2 mg repeated every 5 to 60 minutes until signs and symptoms disappear, up to a maximum of 100 mg in the first 24 hours

Children up to the age of 12 years:

The usual intramuscular, intravenous or subcutaneous dose in children is 10 mcg/kg (0.01 mg/kg), but generally not exceeding 400 mcg (0.4 mg). If necessary, these doses may be repeated every 4-6 hours.

Cardiac: for advanced cardiac life support: intravenous, 20mcg/kg (0.02 mg/kg) with a minimum dose of 10 mcg (0.01 mg) repeated at 5 minute intervals, to a maximum dose of 100 mcg (0.1 mg).

Premedication before anaesthesia: intramuscular or subcutaneous; 30-60 minutes before surgery.

Up to 3 kg -100 mcg (0.1 mg)

7 - 9 kg - 200 mcg (0.2mg)

12 -16 kg - 300 mcg (0.3mg)

Over 20 kg - as for adults.

To control the muscarinic side effects of neostigmine: intravenous; neonates, infants and children - 20 mcg/kg (0.02 mg/kg). Maximum dosage 600 mcg.

Anticholinesterase poisoning: intramuscular or intravenous, 50 mcg/kg (0.05 mg/kg) every 10-30 minutes until muscarinic signs and symptoms disappear.

2.7 Overdose: Symptoms: marked dryness of the mouth accompanied by a burning sensation, difficulty in swallowing, pronounced photophobia, flushing and dryness of the skin, raised body temperature, rash, nausea, vomiting, tachycardia and hypertension. Restlessness, tremor, confusion, excitement, hallucinations and delirium may result from CNS stimulation; this is followed by increasing drowsiness, stupor and general central depression terminating in death from circulatory and respiratory failure. Treatment: In severe cases, physostigmine, 1 to 4 mg, should be administered intravenously, intramuscularly or subcutaneously, the dose may be repeated if necessary since it is rapidly eliminated from the body. Diazepam may be administered for sedation of the delirious patient but the risk of central depression occurring late in the course of atropine poisoning contraindicates large doses of sedative. An adequate airway should be maintained and respiratory failure may be treated with oxygen and carbon dioxide inhalation. Fever is reduced by the application of cold packs or sponging with tepid water. Adequate fluid intake is important. Urethral catheterisation may be necessary. If photophobia is present or likely, the patient should be nursed in a darkened room.

2.8 Undesirable effects: Adverse effects are dose-related and usually reversible when therapy is discontinued. In relatively small doses, atropine reduces salivary, bronchial and sweat secretions, dry mouth and anhidrosis may develop, these effects being intensified as the dosage is increased. Reduced bronchial secretion may cause dehydration of residual secretion and consequent formation of thick bronchial plugs that are difficult to eject from the respiratory tract.

Larger doses dilate the pupil and inhibit accommodation of the eye, and block vagal impulses with consequent increase in heart rate with possible atrial arrhythmias, A-V dissociation and multiple ventricular ectopics; parasympathetic control of the urinary bladder and gastrointestinal tract is inhibited, causing urinary retention and constipation. Further increase in dosage inhibits gastric secretion. Anaphylaxis, urticaria and rash occasionally progressing to exfoliation may develop in some patients. Other effects include hallucinations, increased ocular tension, loss of taste, headache, nervousness, drowsiness, weakness, dizziness, flushing, insomnia, nausea, vomiting and bloated feeling. Mental confusion and/or excitement may occur especially in the elderly.

2.9 Expiry date: It is indicated on the label of each ampoule and on the outer carton. In case this date has passed, do not use this medicine.

2.10 Special precautions for storage: Store below 30°C protected from light

2.11 Date of revision of the text: July 2022 .

3. GENERAL INFORMATION FOR THE RATIONAL USE OF MEDICINES

- Read this entire leaflet carefully before you start taking this medicine. It contains important information for you.
- Keep this leaflet. You may need to read it again
- This medicine has been prescribed for you. Do not pass it on to others. It may harm them, even if their symptoms are the same as yours.
- If you need more information or advice about your medicine, ask your doctor or pharmacist
- Always take this medicine exactly as your doctor has told you in order to get the best results from it.
- If any of the side effects gets serious, or if you notice any side effects not listed in this leaflet please tell your doctor or pharmacist.
- Do not keep your medicines in bathroom medicine cabinets, because heat and humidity can affect their quality. Always keep your medicines in the original container and in a cool and dry place.
- Do not keep medicines that you no longer use or that have already expired.
- Keep all medicines out of the reach and sight of children

4. GENERAL CLASSIFICATION FOR SUPPLY

This medicine is subjected to medical prescription Reg. No.: 10042/20-12-2004



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